

رئيس
المجلس الطبي المتخصص

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Name: بسمه علي ماهر حسن National ID: 28506230101023	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m350

Female	النوع	بسمة على ماهر	المريض
القاهرة	العنوان	32 Y	من
19/01/2020 15:38	تاريخ استلام العينة		تاريخ أخذ العينة
23/01/2020 11:33	تاريخ التقرير	صحة المرأة	التحليل
		صحة المرأة	

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.

Sample recieved was treated by : -VENTANA anti-HER2/neu. (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Sample recieved was treated by :- CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result :

ER: moderate nuclear staining of > 60 % of tumour cells(core biopsy)

PR: moderate nuclear staining of > 60 % of tumour cells(core biopsy)

HER2/neu protein over expression : No membrane staining of tumor cells (core biopsy)

Ki67 :positive nuclear staining of about > 30 % tumour cells (core biopsy)

Referai Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 7/8)

PR: POSITIVE (SCORE 7/8)

HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE

#KI67: THE PROLIFERATITON INDEX OF THE TUMOUR IS ABOUT > 30 %

Pathologist:

د/تريزا ماهر

Lab Director:

د/تريزا ماهر

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يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Page 1 of 1



عزينة د. لؤي

امانة المراكز الطبية المتخصصة
مركز أورام دار السلام
اداره المعمل
قسم الباثولوجي

Pathology Report

Name: بسمة علي ماهر

Age: 35 years.

Clinical data: Right breast cancer, post chemotherapy.

Specimen type: Right wide local excision & sentinel lymph nodes.

Date: 19/8/2020

Lab. Number: 1162

Hospital Number: 1353/2020

Gross Description:

Two specimens were received:

Right wide local excision specimen: breast tissue measured; 7x6x2.5 cm, covering skin ellipse; 4.5x2 cm.

Cut sections revealed an ill-defined rubbery greyish white tumor bed site; 3x1.5x1 cm, 3cm away from skin & 1cm away from deep margin, just below the lateral margin; (least margin).

2) Three nodules; up to 1cm.

Microscopic Examination:

Histopathological examination of sections prepared revealed breast tissue, showed residual malignant invasive tumor, formed of cords and trabeculae of moderately anaplastic ductal epithelial cells, with desmoplastic stroma.

Inconspicuous insitu component.

No perineural invasion or vascular tumor emboli.

Free resection margins.

Free sentinel lymph nodes (0/3).

Diagnosis:

Right breast wide local excision & sentinel lymph nodes, post chemotherapy;

Residual invasive duct carcinoma GII.

Inconspicuous insitu.

No perineural invasion or vascular tumor emboli.

Free resection margins. ✓

Free sentinel lymph nodes.

Stage; ypT2 ypN0. ✓

~~Prof. Dr. Amal Haridy~~
Dr. Enas Mohamed Ali

~~Prof. Dr. Essam Ayad~~



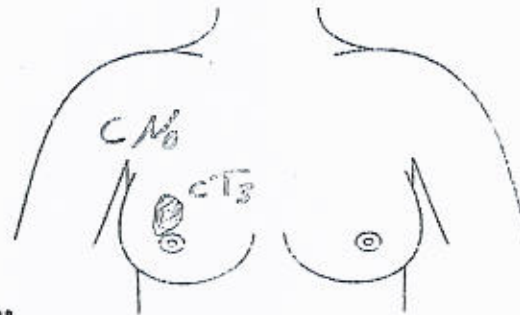
Name: سعد علي طاهر
Age: 32y
No.:

16/3/2020

MDT FORM

Clinical data:

EDC, G II
ER: 7/8
PR: 7/8
Hes-2: -ve
Ki-67: >30%



Reason of discussion:

CT chest, ALN nodes → Pn
- Multic. by CESM
Eche: EF = 45%

MDT Outcome and Plan:

- Sonar-guided Clipping (requested) - Bone scan
- Hes-2: -ve then no evaluation for
possibility of BCS + S/R "over localization"

Signature: : AC → Taxatini

request Bone scan & 1st Gch

16/3/2020

G S S

TAR - Zolater

H 248



Ministry of health
Central health laboratories
Pathology department

Report Number: 46

Histopathology report

Female :	النوع	بسمة على ماهر حسن	اسم المريض :
القاهرة :	العنوان	32 Y	السن :
09/01/2020 13:32 :	تاريخ استلام العينة		تاريخ اخذ العينة :
21/01/2020 13:48 :	تاريخ التقرير		طالب التحليل :
		صحة المرأة	عناية
		صحة المرأة	

Primary sample :

breast core biopsy

comment :

Gross examination :

3 cores of G.W.T. (T.I)

Microscopic examination :

section examined revealed fibrocollagenous tissue infiltrated by malignant growth displaying the morphology of infiltrating duct carcinoma.

Referral Lab :

Comment :

Diagnosis :

BREAST CORES :- THE PICTURE IS COMPATIBLE WITH INFILTRATING DUCT CARCINOMA , GII

Pathologist:

Dr. Jehan Aziz

Lab Director: د/ تيرزا ماهر

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يتم الاحتفاظ بالعينة لمدة 30 يوم من تاريخ استلامها

Name: BASMA ALY

Date: 11-02-2020

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS: -**

Chest:

- Focal areas of right breast parenchymal distortion and non-mass enhancement.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus with left adnexal small cystic changes.
- Reviewing skeletal parts revealed no lytic or sclerotic osseous lesions.

OPINION:

- Focal areas of right breast parenchymal distortion and non-mass enhancement.
- Essentially normal CT study of the chest.
- No CT evidence of hepatic or osseous metastatic disease.

DR. Ibrahim Elsedawy, M.Sc

Dr.m.gomaa.md

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Basma Ali Maher Hassan Ali Al Kholy	Date : 31/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 1353/2020	Pat. ID: 16932

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Right breast conserving therapy

Report:

- Predominantly fatty parenchyma (ACR a)

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse mild edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NGL, Cairo University
31/12/2024 12:46 PM